8956 600 600 96 06 06 3461 / 2 / 3 info@truscan.in
#87, Manvi Archade, 2nd Main Road, W.G.H.S. Layout, J.P. Nagar 7th Phase
Near Brigade Millennium Apartments, Bengaluru, Karnataka - 560078, India.

TruScan Diagnostics

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : EDTA

Billing Time : Jul 17, 2026, 05:24 p.m.
Collection Time : Jul 18, 2026, 07:54 a.m.
Reporting Time : Jul 18, 2026, 10:58 a.m.
Sample ID : 018619826

COMPLETE BLOOD COUNT (CBC) [AUTOMATED]

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
Hemoglobin (Hb) Method : Colorimetric method Sample Type : EDTA	15		13.5 - 18.0	gm/dL
Erythrocyte (RBC) Count Method : Electrical Impedence Sample Type : EDTA	5.1		4.7 - 6.0	mil/cu.mm
Packed Cell Volume (PCV) Method : Calculated Sample Type : EDTA	45		42 - 52	%
Mean Cell Volume (MCV) Method : Calculated Sample Type : EDTA	88.24		78 - 100	fL
Mean Cell Haemoglobin (MCH) Method : Calculated Sample Type : EDTA	29.41		27 - 31	pg
Mean Corpuscular Hb Concn. (MCHC) Method : Calculated Sample Type : EDTA	33.33		32 - 36	gm/dL
Red Cell Distribution Width (RDW - CV) Method : Calculated Sample Type : EDTA	12.8		11.5 - 14.0	%
Total Leucocytes (WBC) Count Method : Flow cytometry by laser Sample Type : EDTA	6040		4000-11000	cell/cu.mm
Neutrophils Method : Flow cytometry by laser Sample Type : EDTA	50.8		40-75	%
Lymphocytes Method : Flow cytometry by laser Sample Type : EDTA	38		22-45	%
Monocytes Method : Flow cytometry by laser Sample Type : EDTA	7.3		0-10	%
Eosinophils Method : Flow cytometry by laser Sample Type : EDTA	3.5		0-6	%
Basophils Method : Flow cytometry by laser Sample Type : EDTA	0.40		0-1	%
Absolute Neutrophil Count Method : Calculated Sample Type : EDTA	3070		2000-7000	cells/c.mm

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Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
Absolute Lymphocyte Count Method : Calculated Sample Type : EDTA	2300		1000-3000	cells/c.mm
Absolute Monocyte Count Method : Calculated Sample Type : EDTA	440		200-1000	cells/c.mm
Absolute Eosinophil Count Method : Calculated Sample Type : EDTA	210		20-500	cells/c.mm
Platelet Count Method : Electrical Impedance Sample Type : EDTA	230		150 -450	10^3/ul
Mean Platelet Volume (MPV) Method : Calculated Sample Type : EDTA	9.1		7.2 - 11.7	fL
PCT Method : Calculated Sample Type : EDTA	**0.210**	**L**	0.220 - 0.300	%
PDW Method : Calculated Sample Type : EDTA	16.4		9.0 - 17.0	%

Tests done on Automated Five Part Cell Counter. (WBC, RBC,Platelet count by impedance method, colorimetric method for Hemoglobin, WBC differential by flow cytometry using laser technology other parameters are calculated). All Abnormal Haemograms are reviewed confirmed microscopically.

END OF REPORT

Kavya B

Dr. Krithika Prasad
Consultant Pathologist
KMC NO 82886

Report Saved By : Kavya B

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : Serum

Billing Time : Jul 17, 2026, 05:24 p.m.
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Sample ID : 018619826

Lipid Profile

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
Cholesterol-Total Method : CHOD-PAP Sample Type : Serum	222.00	H	Desirable: <200 Borderline high: 200-239 High: ≥240 Reference: NCEP ATP 111	mg/dL
Triglycerides Method : GPO- PAP Trinder's Sample Type : Serum	127.00		Normal: <150 Borderline high: 150-199 High: 200-499 Very high: ≥500 Reference: NCEP ATP 111	mg/dL
Cholesterol-HDL Direct Method : PVS-PEGME Sample Type : Serum	53.30		Low: <35 High: ≥79.5 Reference: NCEP ATP 111	mg/dL
LDL Cholesterol Method : Calculated Sample Type : Serum	143.30		Optimal: <100 Near optimal/Above optimal: 100-129 Borderline high: 130-159 High: 160-189 Very high: ≥190 Reference: NCEP ATP 111	mg/dL
Non - HDL Cholesterol, Serum Method : Calculated Sample Type : Serum	168.70		Desirable: < 130 mg/dL Borderline High: 130-159mg/dL High: 160-189 mg/dL Very High: > or = 190 mg/dL	mg/dL
VLDL Cholesterol Method : Calculated Sample Type : Serum	25.40		<30	mg/dL
CHOL/HDL RATIO Method : Calculated Sample Type : Serum	4.17		3.5 - 5.0	Ratio
LDL/HDL RATIO Method : Calculated Sample Type : Serum	2.69		Desirable / low risk - 0.5 -3.0 Low/ Moderate risk - 3.0- 6.0 Elevated / High risk - > 6.0	Ratio
HDL/LDL RATIO Method : Calculated Sample Type : Serum	0.37		Desirable / low risk - 0.5 -3.0 Low/ Moderate risk - 3.0- 6.0 Elevated / High risk - > 6.0	Ratio

Note: 8-10 hours fasting sample is required.

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : Serum

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Sample ID : 018619826

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)

END OF REPORT

Kavya B

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KMC NO 82886

Report Saved By : Kavya B

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
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Referral : SELF
Sample Type : EDTA

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Sample ID : 018619826

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
HBA1C				
Glyco Hb (HbA1C) Method : HPLC Sample Type : EDTA	5.7		Normal: <5.7 Prediabetes: >5.7 -<6.4 Diabetes: - ≥6.5	%
Estimated Average Glucose : Method : Calculated Sample Type : EDTA	116.89		- - 140	mg/dL

Interpretations

1. HbA1C has been endorsed by clinical groups and American Diabetes Association guidelines 2017 for diagnosing diabetes using a cut off point of 6.5%
2. Low glycated haemoglobin in a non diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia (especially severe iron deficiency and haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
3. In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control.

Excellent control-6-7 %
Fair to Good control – 7-8 %
Unsatisfactory control – 8 to 10 %
Poor Control – More than 10 %

END OF REPORT

Kavya B

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Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : Serum

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Sample ID : 018619826

LIVER FUNCTION TEST (LFT)

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
Bilirubin - Total Method : DCA Sample Type : Serum	0.84		0.1 - 1.0	mg/dL
Bilirubin - Direct Method : DCA Sample Type : Serum	0.20		< 0.3	mg/dL
Bilirubin - Indirect Method : Calculated Sample Type : Serum	0.64		0.1 - 1.0	mg/dL
SGOT (AST) Method : IFCC Kinetic Sample Type : Serum	33.00		< 35	U/L
SGPT (ALT) Method : IFCC kinetic Sample Type : Serum	23.00		< 40	U/L
Alkaline Phosphatase-ALP Method : pNPP-AMP Buffer Sample Type : Serum	58.00		53-128	U/L
Total Protein Method : Biuret Sample Type : Serum	6.80		Newborn: 4.6-7.0 1week: 4.4-7.6 7months-1year: 5.1-7.3 1-2years: 5.6-7.5 2-18 years: 6.0-8.0 Adults: 6.4-8.3	g/dL
Albumin Method : BCG Sample Type : Serum	4.30		0-4 days: 2.8-4.4 4 days-14 years: 3.8-5.4 14-18 years: 3.2-4.5 Adult (18-60 years): 3.5-5.2 60-90 years: 3.2-4.6 >90 years: 2.9-4.5	g/dL
Globulin Method : Calculated Sample Type : Serum	2.50		1.8 - 3.6	g/dL
A/G Ratio Method : Calculated Sample Type : Serum	1.72		1.2 - 2.2	ratio
GGT-Gamma Glutamyl Transpeptidase Method : IFCC Kinetic Sample Type : Serum	22.30		11 -50	U/L

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
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Source : DIRECT
Referral : SELF
Sample Type : Serum

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Sample ID : 018619826

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)

END OF REPORT

Kavya B

Dr. Krithika Prasad
Consultant Pathologist
KMC NO 82886

Report Saved By : Kavya B

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : SERUM

Billing Time : Jul 17, 2026, 05:24 p.m.
Collection Time : Jul 18, 2026, 07:54 a.m.
Reporting Time : Jul 18, 2026, 10:58 a.m.
Sample ID : 018619826

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)

THYROID FUNCTION TESTS (TFT)

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
T3-Total* Method : ECLIA Sample Type : Serum	0.87		0.87 - 1.78	ng/ml
TOTAL T4 Method : ECLIA Sample Type : Serum	5.72		4.61 - 14.94	ug/dL
TSH Method : ECLIA Sample Type : Serum	2.94		0.38 - 5.33	uIU/ml

Interpretation

It is recommended to interpret serum TSH levels with thyroid hormone levels (especially T4 levels) taking into consideration the clinical status of patient. Pitfalls in the interpretation of the serum TSH alone are in patients with recent treatment for thyrotoxicosis, non-thyroidal illness(acute severe illness or chronic illness), central hypothyroidism, confounding medications.

Condition	TSH	T4	T3
Primary Hypothyroidism	Increased	Low	Normal /Low
Subclinical Hypothyroidism	Increased	Normal	Normal
Primary Hyperthyroidism	Decreased	Increased	Increased
T3 Toxicosis	Decreased	Normal	Increased
Subclinical Hyperthyroidism	Decreased	Normal	Normal
Central Hyperthyroidism/ Thyroid Hormone Resistance	Increased /Normal	Increased	Increased
Central Hypothyroidism / Non Thyroidal Illness	Decreased /Normal	Decreased	Decreased

END OF REPORT

Kavya B

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Report Saved By : Kavya B

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : Serum

Billing Time : Jul 17, 2026, 05:24 p.m.
Collection Time : Jul 18, 2026, 07:55 a.m.
Reporting Time : Jul 18, 2026, 11:16 a.m.
Sample ID : 018619826

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
RENAL PROFILE (RFT)				
Urea Method : UV KINETIC Sample Type : Serum	25.00		1 hr-1month : 8.6-25.7 1 month - 1 year : 10.7-38.5 1 year - 60 years : 12.84-42.8 >60 years : 17.12-49.22	mg/dL
Blood Urea Nitrogen-BUN Method : Calculated Sample Type : Serum	11.68		1 hr - 1 month: 4-12 1 month - 1 year : 5-18 1 year - 60 years : 6-20 >60 years : 8-23	mg/dL
Creatinine Method : JAFFE Sample Type : Serum	1.05		0.66 -1.25	mg/dL
Sodium Method : Indirect ISE Sample Type : Serum	135		135 - 145	mmol/L
Potassium Method : Indirect ISE Sample Type : Serum	4.0		3.5 - 5.1	mmol/L
Chloride Method : Indirect ISE Sample Type : Serum	96		96-107	mmol/L

END OF REPORT

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TruScan Diagnostics

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : SERUM

Billing Time : Jul 17, 2026, 05:24 p.m.
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Sample ID : 018619826

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
VITAMIN B12				
VIT B12 Method : ECLIA Sample Type : Serum	644		197-721	pg/ml

Interpretation:

Vitamin B12, also known as cyanocobalamin, is a water soluble vitamin that is required for the maturation of erythrocytes and coenzyme form for more than 12 different enzyme systems. Groupsat risk for vitamin B12 deficiency include those

(1) older than 65 years of age (2) with malabsorption(3) who are vegetarians (4) with autoimmune disorders(5) taking prescribedmedication known to interfere with vitamin absorption or metabolism, including nitrous oxide, phenytoin, dihydrofolate reductase inhibitors, metformin, and proton pump inhibitors(6) infants with suspected metabolic disorders.

The most common cause of Vitamin B12 deficiency is pernicious anemia. Deficiency of Vitamin B12 is associated with megaloblastic anemia and neuropathy. Excess Vitamin B12 is excreted in urine. No adverse effects have been associated with excess vitamin B12 intake from food or supplements in healthy people.

END OF REPORT

Kavya B

Dr. Krithika Prasad
Consultant Pathologist
KMC NO 82886

Report Saved By : Kavya B

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : SERUM

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Test Description	Value(s)	Flag	Biological Reference Interval		Unit(s)

VITAMIN D TOTAL

Test Description	Value(s)	Flag	Biological Reference Interval		Unit(s)
VIT D **Method : ECLIA** **Sample Type : Serum**	50.07		Deficient Insufficient Sufficient Excess Intoxiation	< 20 20 to <30 30 - 100 > 100 >150	ng/mL

Interpretation:

Useful for :

Diagnosis of vitamin D deficiency .

Differential diagnosis of causes of rickets and Osteomalacia . Monitoring vitamin D replacement therapy . Diagnosis of hypervitaminosis D .

Vitamin D levels may vary according to factors such as geography, season, or the patient’s health, diet, age, ethnic origin, use of vitamin D supplementation or environment.

Some potential interfering substances like rheumatoid factor, endogenous alkaline phosphatase, fibrin, and proteins capable of binding to alkaline phosphatase in the patient sample may cause erroneous results in immunoassays.Carefully evaluate the results of patients suspected of having these types of interferences.

END OF REPORT

Kavya B

Dr. Krithika Prasad
Consultant Pathologist
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TruScan Diagnostics

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : Urine

Billing Time : Jul 17, 2026, 05:24 p.m.
Collection Time : Jul 18, 2026, 11:35 a.m.
Reporting Time : Jul 18, 2026, 11:36 a.m.
Sample ID : 018619826

COMPLETE URINE ANALYSIS

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
Urine Volume	20			ml
Urine Colour	Pale Yellow		Pale Yellow	
Urine Transparency (Appearance)	Clear		Clear	
Urine Deposit	Absent		Absent	
Urine Reaction (pH)	6.5		4.5 - 8	
Urine Specific Gravity	1.015		1.010 - 1.030	
Chemical Examination (Automated Dipstick Method) Urine				
Urine Glucose (sugar)	Absent		Absent	
Urine Protein (Albumin)	Absent		Absent	
Urine Ketones (Acetone)	Absent		Absent	
Urine Blood	Absent		Absent	
Urine Bile pigments	Absent		Absent	
Urine Bile salt	Normal		Normal	
Urine Nitrite	Absent		Absent	
Microscopic Examination Urine				
Pus Cells (WBCs)	2-3		0 - 4	/hpf
Epithelial Cells	1-2		0 - 2	/hpf
Red blood Cells	Absent		Absent	/hpf
Crystals	Absent		Absent	
Cast	Absent		Absent	
Trichomonas Vaginalis	Absent		Absent	
Yeast Cells	Absent		Absent	
Amorphous deposits	Absent		Absent	
Bacteria	Absent		Absent	

END OF REPORT

Hanumantharaju

Dr. Krithika Prasad
Consultant Pathologist
KMC NO 82886

Report Saved By : Hanumantharaju

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TruScan Diagnostics

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
Patient ID : 244414
Source : DIRECT
Referral : SELF
Sample Type : SERUM

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Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)

TESTOSTERONE TOTAL

Test Description	Value(s)	Flag	Biological Reference Interval	Unit(s)
Total Testosterone Method : ECLIA Sample Type : Serum	520.2		175 - 781	ng/dL

Increased Levels (Male)	**Decreased Levels (Male)**
Idiopathic sexual precocity	Klinfelter syndrome
Pinealoma	Cryptorchidism
Encephalitis	Primary and secondary hypogonadism
Congenital adrenal hyperplasia	Trisomy 21 (down syndrome)
Adrenocortical tumor	Orchidectomy
Testicular or extragonadal tumor	Hepatic cirrhosis
Testosterone resistance syndromes	
Increased Levels (Female)	**Decreased Levels (Female)**
Ovarian tumor	
Adrenal tumor	
Congenital adrenocortical hyperplasia	
Trophoblastic tumor	
Polycystic ovaries	
Idiopathic hirsutism	

Interfering Factors:

- Drugs that may cause increased testosterone levels include anticonvulsants, barbiturates, estrogens and oral contraceptives.
- Drugs that may cause decreased testosterone levels include alcohol, androgens, dexamethasone, diethylstilbestrol,? digoxin, ketoconazole, phenotiazine, spironolactone and steroids.

END OF REPORT

Kavya B

Dr. Krithika Prasad
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TruScan Diagnostics

Patient Name : MR. SRIKRISHNA GOPALAKRISHNAN
Age / Gender : 48 years / Male
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